NCCP 00483 · clinician draft

Nivolumab · every 2 weeks · previously treated NSCLC

Previously treated NSCLC Previously treated locally advanced or metastatic non-small-cell lung cancer (NSCLC). This patient suite is lung-focused; other shared protocol indications need their own counselling.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Nivolumab every 2 weeks

Active medicines: nivolumab

Emetogenic risk: Minimal

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- Minimal emetic risk; no routine antiemetic prophylaxis or premedication. Prescribe rescue/support individually.
- Immune alert card and immediate symptom reporting.
- Blood count, kidney/liver tests and glucose before every 2-week cycle; thyroid once a month; cortisol if clinically indicated. Review cancer progression before cycle 8. Continue immune-toxicity monitoring for at least 5 months after the last dose.
- Effective contraception during treatment and at least 5 months afterwards; specialist breastfeeding discussion.
- Current NCIS medical-oncology guidance does not create a routine cytotoxic prescription for minimal-risk nivolumab monotherapy.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00483 v13a · pNCCP 00483 pp8–9 · NCCP 00483 v13a supportive care

Sources checked 2026-10-29 · SACTCheck v0.84.27 · Regimen-level drafts; prescriber and oncology-pharmacy verification required